NURSING DEPARTMENT

Policy Name : Admission of an Infant in NICU

Policy Number : ABCD-ND 1/3 (8)

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1. **Purpose**
    a. To determine baseline data of the infant's condition, to provide prompt therapy and life saving measures and perform assessment, planning and intervention that will require immediate treatment
2. **Scope**
    a. Case accepted and admission ordered by the Consultant.
    b. Proper documentation of the procedure and report for any abnormality
3. **Definition**
    a. Admission of an infant to NICU: A neonate should be admitted to NICU when the baby is unstable and need high dependency care continuously from the medical and nursing staff
4. **Abbreviation**:
    a. NICU - Neonatal Intensive Care Unit
5. **Policy Statement**
    a. All admissions must have a written physician's order.
    b. Emergency equipment, as in ventilator must always be ready for admission.
    c. Admission to NICU is categorized as;
        i. Infants requiring cardiopulmonary resuscitation.
        ii. Infants requiring prolong mechanical ventilation
        iii. Infants who are critically ill, preferably clean cases.
        iv. Seriously ill infants delivered from delivery room without suspected maternal infection.
        v. Infants requiring close observation and monitoring, infants with metabolic disorders.
        vi. Infants below 37 weeks of gestational age.
        vii. Infants with life threatening surgery.
        viii. High risk infants requiring extensive sophisticated equipment.
6. **Procedure and Rationale:**

Procedure	Rationale
1. Verifies physician's order (drug, dosage, route and time intervals)	For safe drug administration and patient's safety.
2. Admit infant without delay in order to administer immediate treatment.	Delays in the treatment possess multitude consequences especially for neonates.
3. Place the infant under preheated radiant warmer.	To stabilize the patient's temperature and avoid hypothermia.
4. Wash hands and dry infants.	To reduce the transmission of microorganism and dry hands to prevent further hypothermia .

5. Clear the airway by chin lift and head tilt maneuver.	To remove secretions obstructing the airway.
6. Prepare medication for only one patient at a time to prevent errors during preparation. Assess for adequacy of respiratory effort. 6.1. Stimulate by flicking the sole and rubbing back gently. 6.2. Initiate breathing by using bag and mask technique and prepare for possible intubation. 6.3. Apply oxygen via head box if there is slight perip cyanosis	Respiratory activity is assessed by observing chest movement or listening for breath sounds. lf there is no respiratory effort or the effort is poor, the infants need respiratory assistance by either manual stimulation or positive pressure ventilation.
7. Monitor closely for respiratory or cardiac complications. 7.1. Respiration above 60/min, gently suction mouth and nose. 7.2. Expiratory grunting, retractions, chest lag and nasal flaring. 7.3. Watch for cyanosis, increased or irregular heart rate. 7.4. Evaluate muscle tone and activity.	A respiratory rate >60/min implies the inability to generate an adequate tidal volume and may persist during mechanical ventilation. intercostal, subcostal and sterna retractions are present in conditions of decreased lung compliance or increased airway resistance. Central cyanosis indicates hypoxemia. Cyanosis is difficult to appreciate in the presence of anemia and oxygen supplementation is needed. Patient with compromised respiratory effort is often hypotonic to fairly active.
8. Position the infant properly to allow easily ventilation. 8.1. Elevate head and trunk. 8.2. Change position from side to side.	The main physiological aims of proper positioning is: to improve oxygenation, respiratory mechanics, to homogenize the pleural pressure gradient, the alveolar inflation and the ventilation distribution, to increase lung volume and reduce the amount of atelectatic regions, to facilitate the drainage of secretions and to reduce ventilator-associated lung injury.
9. Provide oxygen therapy with moisture in the percentage to maintain appropriate blood gas values. 9.1. Use of pulse oximeter to monitor oxygenation.	To overcome drying and cooling of respiratory tract. Use of pulse oximeter to regularly monitor patient's oxygen saturation and heart rate.
10. Observe scrupulous hand washing. Minimize infant's contact with unsterile equipment.	To protect infant from infection and cross contamination.

11. Provide psychological support to the infant's parents.	To alleviate anxiety and fear and understand infant's condition.
12. Prepare for ventilation.	For any admission in the unit, it is a routine that ventilators are always made available in case of intubation.
13. Administer medications carefully as ordered.	All medications should be administered slowly to prevent the occurrence of intraventricular hemorrhage and intravenous infiltrations.
14. Collect blood samples for laboratory analysis.	Blood extraction should be done aseptically. Routinely it is done to have baseline data for treatment.
15. Assist the pediatrician in umbilical catheterization of IV fluid administration to initiate fluid therapy.	Especially done for patients developing hypotension and severe hypoglycemia for better access in administering high dextrose fluids.
16. Document the procedure and report any abnormality.	Proper documentation for any legal purposes and evidence of treatment rendered to the patient.

7. **Responsibilities**
    a. Treating physician - To obtain history and to provide a written order for admission and his/her signature and Stamp.
    b. Charge Nurse - To monitor the nurses' performances at the bed side and assist when needed as per institutional policies.
    c. Staff Nurse - To receive and position the baby in PR+. heated radiant warmer, do initial assessment and carry out physician's order.
8. **Documentation**
    a. Pediatric and Neonate Assessment Form (FM-106 PNA)
    b. Physician Progress Notes (FM-009)
    c. Physician Order Sheet (FM-010)
    d. Nurses' Flow Chart (FM-051)
9. **Measurements**
    a. 100% of cases admitted are following admission criteria
    b. Re-admission with the same diagnosis within 48 hours not more than 2%
10. **References**
    a. Medical by laws
    b. CBAHI Standards
    c. Manual of Neonatology, 5th edition

11. **Cross References**
    a. Admission Policy
12. **Attachments**
    N/A
13. **Distribution List**
    a. Pediatric Department
    b. Quality Department
    c. Office of Executive Director General
14. **Review Summary**
    a. Additional Definition
    b. Re-statement of the Purpose
    c. Addition of Procedure and Rationale